Guidelines for Clinical Trials in Cameroon

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FOREWORD

Clinical research aims to improve human health and increase our understanding of human biology, which ultimately leads to significant advances in scientific knowledge. Clinical trials are essential for the development of new drugs and new technique of drug delivery, treatment and diagnosis. It thus contributes to optimizing case management and consequently, the performance of the health system. However, it must be done in compliance with the rules of ethics and protection of participants.

Law No. 2022/008 of April 27, 2022 relating to "Medical Research involving Human Persons in Cameroon", promulgated by the President of the Republic, completed the process of supervising human health research in Cameroon begun since 2002.

Although real progress in the development of the regulation of human health research has been made, there is a need to put in place guidelines to ensure effective protection of participants in clinical research.

This guide entitled "Guidelines for Clinical Trials in Cameroon" is an essential working tool made available to all stakeholders involved in human health research, to enable them to carry out their projects in accordance with best clinical practices and according to recognized international guidelines.

To this end, I invite you to appropriate it and make good use of it for the benefit of clinical research policy as well as the beneficiaries of the conclusions of said research.

THANKS

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LIST OF ACRONYMS AND ABBREVIATIONS

AAR	Administrative Research Authorization
AMM	Marketing Authorization
ANRPh	National Pharmaceutical Regulatory Authority
HAVE YOU	Temporary authorization of use
PCB	Good Clinical Practices
GMP	Good Manufacturing Practices
CIOMS	International Ethical Guidelines for Health Research Involving Human Participants
CNERSH	National Research Ethics Committee for Human Health
DROS	Division of Operational Research in Health
ICH	International Conference on Harmonization of technical requirements for the registration of medicinal products for human use
IGSPL	General Inspection of Pharmaceutical Services and Laboratories
MINSANTE	Ministry of Public Health
MTA	Improved traditional medicine
DPML	Directorate of Pharmacy, Medicines and Laboratories
MY	Standard operating procedure

CHAPTER I: GENERAL

Section 1: Introduction

The "Guidelines for clinical trials in Cameroon" aim to ensure that clinical trials conducted in Cameroon are designed and conducted according to scientific rigor and ethical standards in accordance with Good Clinical Practices [1, 2]. Compliance with the requirements contained in these guidelines provides the public with confidence that the rights, safety, and well-being of clinical trial participants are protected.

Section 2: Scope of these guidelines

These guidelines:

(1) Establish the legal requirements for the authorization and conduct of clinical trials as defined in the framework law in the field of health [3], and the law relating to medical research involving humans [4].

(2) Apply at all phases of clinical trials relating to drugs and vaccines, medical equipment, health products, the development of preventive, diagnostic and therapeutic procedures whose aim is to improve human health.

(3) Aimed at researchers, the pharmaceutical industry, clinical research organizations and sponsors of clinical trials, whether for academic purposes or for the generation of data, intended to be included in their future application file., for Marketing Authorization (AM) [5].

(4) Provide specific requirements for clinical trials of improved traditional medicines [6].

Section 3: Terms and definitions[1]

Administrative Research Authorization (AAR) [7]: Prior agreement issued by the Ministry of Health to allow a researcher to actually begin their research in the field, after the latter has obtained ethical clearance from the National Ethics Committee for Human Health Research (CNERSH).

National Pharmaceutical Regulatory Authority (NPRA): The NPRA is created by Decision No. 0732 of June 19, 2017 [8]. It is responsible for coordinating and controlling the pharmaceutical sector in order to preserve public health. The NPRA is made up of the following structures:

- The general inspection of pharmaceutical services and laboratories;
- The management of the pharmacy, medicines and laboratories;
- The national drug quality control and expertise laboratory;
- The family health department;
- The health operational research division.

Clinical Trail: Any systematic study of intervention in a volunteer, whether sick or healthy, which includes, among other things, drugs and other biological products, surgical procedures, radiological techniques, devices, behavioral therapies, changes in care protocols, preventive care, in order to evaluate the effects of these interventions on health.

Multicenter trial: Clinical trial carried out according to a single protocol at several different locations and, therefore, by more than one investigator.

Good Clinical Practices (GCP): Standard concerning the design, conduct, effectiveness, monitoring and verification of clinical trials as well as the recording,

[1]The terms defined in these guidelines come mainly from the Guidelines for Good Clinical Practice: integrated addendum to ICH E6(R2), and from the law relating to medical research involving humans in Cameroon. definitions will de facto take on the meanings of any consequential updates to ICH E6 (R2) and the Medical Research Act where necessary.

analysis and presentation of related data and which guarantees the reliability and accuracy of the data and results presented as well as the protection of the rights, integrity and identity of the subjects.

Investigator Brochure: Document made available to the investigator by the sponsor and containing all clinical and non-clinical information relating to the product to be tested during the clinical trial, as well as advice on how to ensure the clinical management of subject's participants in this trial.

Ethical clearance: Opinion given by the Ethics Committee, after its approval of the scientific validity and compliance with ethical principles of the research protocol submitted by the sponsor or investigator.

Health-related personal data: Any information concerning an identified (nominative data) or identifiable person (coded or pseudonymized data) which has a link to their state of health or illness, including genetic data.

Safety and Data Monitoring Committee (Independent Data Monitoring Committee, Monitoring Committee, Data Monitoring Committee): Safety and data monitoring committee which may be established by the sponsor and responsible, on the one hand, for periodically evaluating the progress of a clinical trial, the safety data and the critical results concerning the effectiveness and, on the other hand, to make recommendations concerning the continuation, modification or interruption of a trial.

Confidentiality: Non-disclosure, to persons other than authorized persons, of information exclusive to the promoter or the identity of a subject.

Informed consent: Process by which a subject voluntarily confirms their willingness to participate in a particular trial, after having been informed of all aspects of the trial that may influence their decision. Informed consent is recorded on a written, signed and dated form.

Adverse reaction to a medical product: Any harmful or unexpected reaction caused by any dose of a medical product (see the guidelines for the management of clinical data on drug safety [9]).

Therapeutic incident: Any untoward manifestation of a medical nature in a patient or clinical study subject to whom a pharmaceutical product has been administered and which does not necessarily have a causal link with the treatment.

Serious therapeutic incident or serious adverse effect (reaction) to a medication: Any untoward medical event during the clinical trial which, regardless of the dose of drug, meets at least one of the criteria below:

- results in the death of the subject;
- threatens life;
- requires the hospitalization of the patient or the extension of his hospitalization;
- causes persistent or marked disability or incapacity;
- manifests itself as an anomaly or congenital malformation.

Research ethics: Set of moral, ethical and behavioral rules that apply to anyone carrying out research activities [10].

Inspection: Official examination conducted by a regulatory body of documents, facilities, records, and any other resources that the regulatory body deems relevant to the clinical trial and that may be located at the trial site, the sponsor's or contract research organization's (CRO) facilities, or any other establishment deemed appropriate by the regulatory body.

Investigator: Any person responsible for the scientific conduct of a human health research project.

Principal investigator: The investigator who is responsible for a team of investigators and the overall conduct of a medical research project at one or multiple designated sites.

Trial site: The location where activities related to the trial are conducted.

Modification (amendment) to the protocol: Written description of an official modification or clarification made to the protocol.

Research Product: The pharmaceutical form of an active ingredient (drug), intervention, or placebo being tested or used as a reference product in a clinical trial. It also includes a product that has been authorized for marketing and is used (formulated or packaged) in a manner other than the approved form or for unapproved indications, or to collect additional data regarding an approved use**.**

Promoter: Person, natural or legal entity responsible for launching, managing and financing a clinical trial.

Sponsor-investigator: Person who implements and carries out, alone or with others, a clinical trial, and under whose direct responsibility the research product is administered to a subject. The term is reserved for a real person (e.g., it does not apply to a corporation or organization). The obligations of a sponsor-investigator are both those of a sponsor and those of an investigator.

Research protocol: Dated document, approved by the promoter and by the investigator which integrates successive modifications and describes the justification, objectives, design, method, statistical aspects, organization of the research, as well as ethical issues, the funding and institutional affiliations.

Report on a clinical trial: Written description of a trial conducted on human subjects and relating to a therapeutic, prophylactic or diagnostic agent, are integrated to form a single report: the description, presentation and analysis of clinical and statistical data [11].

Biological material: Human substances from living or deceased persons, fetuses or embryos; or animals.

Legal representative: Natural or legal person authorized under the relevant law to consent, on behalf of a potential subject, to the latter's participation in the clinical trial.

Subject/subject participating in the trial: Person who participates in a clinical trial, whether in the experimental group or in the control group.

Placebo: Inactive substance administered to certain participants in a study for purposes of comparing the effects with those of participants who received the product used.

Standard (reference) treatment: Medication or intervention approved by a competent authority for a specific indication.

Vulnerable subjects: Individuals whose desire to participate in a clinical trial may be unduly influenced by the hope, justified or not, of receiving benefits associated with their participation or the fear of reprisals from more influential members of a hierarchical structure in the event of refusal. Examples include members of a group with a hierarchical structure, such as medical, pharmacology, dental, and nursing students, junior hospital or laboratory employees, pharmaceutical industry members, members of the armed forces, and prisoners. Patients with incurable illnesses, people living in nursing homes, unemployed or poor people, patients facing an emergency, members of ethnic minorities, the homeless, nomads, refugees, minors, and people incapable of giving consent are also vulnerable subjects.

Monitoring: Activity which consists of monitoring the progress of a clinical trial and ensuring that this trial is carried out in accordance with the protocol, standard operating procedures (SOP), good clinical practices (GCP) and applicable regulatory requirements, and that the related data is recorded and presented in accordance with these same directives.

CHAPTER II: PROCEDURES FOR IMPLEMENTING CLINICAL TRIALS

Section 1: Reminder on clinical trials

Clinical trials are classified into 4 phases (phases I to IV). A brief, non-exhaustive description of the different phases is given below [12].

(1) Phase I

Objective: Carry out a preliminary assessment of the safety of the active ingredient, its pharmacokinetic profile and, if possible, pharmacodynamics in humans.

Features: This is the first administration to humans of an active ingredient validated in the preclinical phase. Often carried out in healthy volunteers who are administered reduced doses of said active ingredient.

(2) Phase II

Goals: Demonstrate the existence of therapeutic activity and evaluate the short-term safety of the active ingredient.

Features: Takes place on subjects suffering from or exposed to a disease or condition for which the active ingredient should be indicated. This phase also aims to determine appropriate dosage or treatment regimens and (if possible) to clarify dose-response relationships.

(3) Phase III

Goals: Determine the effectiveness of the active ingredient or intervention by comparing it to other standard and/or experimental treatments. These studies also enable further characterization of the profile of any common adverse reactions and the particular characteristics of the product (e.g. clinically relevant drug interactions, factors leading to differences in effect such as age) or intervention.

Features: Preferably be designed as double-blind randomized trials, but other study types may be acceptable. Generally speaking, the conditions in which these tests are carried out must be as close as possible to the normal conditions of use of the active ingredient or the intervention.

(4) **Phase IV**

Goals: determine long-term adverse effects and measure the effectiveness of a drug or intervention under real conditions of use in the general population.

Features: studies carried out after marketing the product (post-MA) taking into account the indications for which the product was approved. These studies must use the same scientific and ethical standards as those applied in phase I to III studies.

Nota Bene: once a research product has obtained a marketing authorization (MA) in Cameroon, any study concerning it and designed to explore new indications, dosages, methods of administration, combinations, etc., will be considered and treated as a clinical trial.

Section 2: General requirements [4]

(1) The clinical trial of a research product can only be carried out on human beings if and as long as:

- The foreseeable potential risks and disadvantages are medically justifiable and do not exceed the expected benefits for the subject participating in the clinical trial;
- The expected importance of the research product for science and medicine is proven;
- according to the state of scientific knowledge, it is proven that the clinical trial when it concerns a research product corresponding to a genetically modified organism or a combination of genetically modified organisms or containing such organisms, must not induce effects unjustifiable harm to the health of third parties and the environment;

- the participant in the clinical trial, if an adult, has been informed of the nature, importance and implications of the clinical trial and has given written and voluntary informed consent which implies that:
    i. the trial participant or witness and/or legal representative, if applicable, is informed by the principal investigator, or a qualified person designated by the principal investigator;
    ii. the informed consent procedure is carried out according to the rules of Good Clinical Practice;
    iii. the consent given by the trial participant, or the witness and/or legal representative, if applicable, is documented in writing, dated and signed by each of those concerned;
    iv. if the subject participating in the trial is incapable of reading or writing French or English, informed consent is obtained in the presence of at least one literate witness, speaking the language of the latter, but not part of the study team. In this case, the consent form must contain a thumbprint of the trial participant to signify their approval to participate in the clinical trial. The participation of the literate witness in the process of obtaining consent must be documented in writing and the consent form must collect the signature of the witness.
    v. the subject participating in the trial is informed of the purpose of the collection and use of their personal data, in particular medical data; the trial participant will be informed in particular of the fact that, where applicable, the data collected will be transmitted to the sponsor and the competent services of the Ministry of Public Health without revealing the identity of the trial participant and will be made available during any inspection of the trial site;

- the use of the investigational product is indicated in accordance with recent data and scientific discoveries and is intended to prevent the occurrence of an illness or to save the life of the sick person, to restore their health, to relieve their suffering, or
- this drug once developed must directly benefit the group of patients suffering from the same disease as the subject participating in the trial.

(2) Any clinical trial must have a "sponsor", namely a legal or natural person responsible for its initiation and management. This "simple liability" is to be distinguished from questions linked to "civil and criminal liability" relating to damage suffered by patients. The applicable civil and criminal liability regime depends on national laws in this area.

(3) The sponsor, the investigator and all other people involved in carrying out a clinical trial must, when conducting it, satisfy:

- the requirements of Good Clinical Practice as defined in the current ICH guidelines [l]and in the International Ethical Guidelines for Health Research Involving Human Participants [13];
- any other requirement considered relevant by the Ministry of Health of Cameroon.

(4) The clinical trial can only be initiated by the sponsor if the National Ethics Committee for Research in Human Health (CNERSH) has issued an ethical clearance and the Ministry in charge of Public Health has issued an Administrative Research Authorization. based on the conclusion that the expected therapeutic and public health benefits are greater than the risks potentially incurred by the study subjects and the population. The conduct of this clinical trial can only be continued if the risk/benefit ratio remains favorable during the monitoring of the trial.

(5) The request for ethical clearance and the request for Administrative Research Authorization are filed respectively with the National Committee for Research

Ethics for Human Health and the Ministry in charge of Public Health, the request is signed by the sponsor and the principal investigator. When the principal investigator does not have an institutional and/or physical address in Cameroon, the application is signed by the sponsor and the investigator residing in Cameroon.

(6) The request for ethical clearance and the request for Administrative Research Authorization must be made sequentially except in the context of situations duly defined by the Ministry of Public Health such as in cases of Public Health emergency.

(7) The Administrative Research Authorization is valid for the duration of the study specified in the research protocol or in specific cases for a duration duly defined by the Ministry in charge of health.

(8) The ethical clearance is valid for a period of two (02) years, renewable, where applicable, the ethical clearance is valid for the period specified by the National Ethics Committee [14],

(9) Obtaining ethical clearance is subject to submission of a complete file to the National Ethics Committee for Human Health Research. This file includes the following documents:

- a request written in French or English, stamped at the current rate, signed by the national principal investigator and addressed to the Chairman of the Committee;
- a paginated research protocol written either in French or in English, signed by all investigators, and presented according to the plan defined by the CNERSH;
- the annexes: information notice and consent form written in English and French, data collection tools, curriculum vitae dated and signed by the principal investigator and co-investigators of the research team;

- the different ethical clearances where applicable;
- the agreement of the head of the health structure(s) where the research will be carried out;
- a letter of commitment to make the research results accessible to the research population and to comply with all other requirements provided for by the regulations in force;
- a certificate of payment of the research protocol evaluation fees;
- the information leaflet of the investigational product;
- an investigator's brochure;
- a valid insurance policy covering possible damages to subjects and investigators;
- when the research project involves a transfer of biological material and an exchange of data collected between several national or international institutions, the file includes, in addition to the documents mentioned above:
    i. the biological data transfer agreement or "Material Transfer Agreement";
    ii. the study data sharing agreement or Data Sharing Agreement;
- a results dissemination plan that targets research participants, local and national health authorities, the relevant Ethics Committee as well as scientific peers;
- the budget relating to the implementation of the dissemination plan.

(10) The Administrative Research Authorization (AAR) application file submitted to the Ministry of Public Health must include the following documents:

- a letter of request stamped at the current rate addressed to the Minister in charge of Public Health and including the list of documents submitted with their version numbers and dates;

- the duly completed AAR request checklist for a clinical trial;
- the clinical trial protocol dated and signed by the Principal Investigator, approved by the sponsor or promoter, and including the address of the natural or legal person submitting the request (postal box, fax, telephone, email address and the headquarters of the institution or research center);
- the list of planned clinical trial sites and the number of subjects to be included on the sites located in Cameroon;
- a description of the site(s) to host the clinical trial in Cameroon indicating their suitability for the clinical trial concerned;
- the research participant's information sheet, procedure and informed consent form(s) and assent form in the event that minors are to participate in the clinical trial in French and English;
- the investigator's brochure;
- information on the research product if it already benefits from a Marketing Authorization;
- Marketing Authorization (for phase 4 studies);
- where applicable, the summary of previous clinical trials with the investigational medical product(s);
- the investigational product file including preclinical data (if applicable);
- where applicable, electronic copies of key peer-reviewed publications in accordance with ICMJE recommendations [15];
- the copy(ies) of recruitment advertisements (if applicable) and questionnaires;
- the content of the labeling of research products;
- product information and certificate of analysis for concomitant medications and emergency medications;
- the certified copy of the Good Manufacturing Practices Certificate (GMP) issued by the competent authority of the country where

the investigational product is produced, translated into English and/or French;

- the certificate(s) of analysis of the research product(s) in Cameroon;
- the sworn declaration signed by the principal investigator attesting that he undertakes to respect the GCP as well as the terms of monitoring of the Ministry in charge of Public Health and to provide a restitution of the work at the end of the project;
- the plan for disseminating the results of the work;
- the prior agreement signed by the head of each health structure/institution where the study will be implemented;
- the dated and signed Curriculum vitae of each of the investigators taking part in the conduct of the clinical trial, (National Principal Investigator, principal investigators and/or co-investigators, study coordinator, regional and local monitor, contract research affiliate, etc.);
- the budget signed between the sponsor and the national principal investigator;
- the declaration on honor signed by the monitor(s) to respect the GCP as well as the monitoring terms of the Ministry in charge of Public Health;
- the sworn declaration signed by the promoter/sponsor-investigator of the trial attesting that he is aware of the protocol and that he will comply with good clinical practices as determined by the Ministry in charge of Public Health in the conduct of the test;
- proof of registration of the clinical trial in a main register accessible online;
- the insurance policy valid during the period of the clinical trial covering possible damages to subjects and investigators;

- the Sponsor's commitment to caring for subjects who are victims of incidents/accidents linked to their participation in the clinical trial and which may occur in the medium or long term;
- GCP certificates of all investigators:
    i. proof of registration of the principal investigator and the national co-investigator(s) with a professional statutory body such as the order of doctors or the order of pharmacists (if applicable);
    ii. proof of residence in Cameroon of the national principal investigator;
    iii. the detailed budget of the clinical trial, the sources of financing and the full address of the promoter and sponsor of the project;
    iv. the ethical clearance issued by the CNERSH in the event of a sequential procedure;
    v. the list, terms of reference and curriculum vitae of the members of the Data Safety Monitoring Board;
    vi. when the research is multicenter or when it provides for the transfer of data or biological research material between the issuing institution and the recipient institution outside Cameroon, the following documents will also be required:
    - ✓ the data transfer agreement (Data Sharing Agreement);
    - ✓ the material transfer agreement (Material Transfer Agreement).

(11) Any substantial modification of the documents of a clinical trial, the terms of this trial and/or the research product having received ethical clearance and an Administrative Research Authorization constitutes an amendment. It must be submitted by the principal investigator to the CNERSH and to the Ministry in charge of Public Health with the fees payable for the evaluation.

(12) In the event of a health emergency situation including natural or accidental disasters, epidemics and humanitarian crises, the Ministry in charge of Public Health can:

•apply an accelerated or emergency procedure for the evaluation and authorization of clinical trials;

• grant exemptions for certain constituent elements of the clinical trial application submission file.

Section 3: Special Requirements

Special requirements concern clinical trials conducted:

i. in medical emergency situations;
ii. on minor matters;
iii. on incapable adults;
iv. on people who have lost the capacity for discernment after consent;
v. on the pregnant woman, the embryo, or the fetus in vivo;
vi. on the embryo in vitro.

(1) Clinical trials carried out in medical emergency situations

• Clinical trials in medical emergency situations will be evaluated by the competent ethics committee and supervised by the Ministry of Public Health. This will depend on each clinical trial and the type of research product concerned.

• In a medical emergency situation where informed consent cannot be obtained before the subject's participation in the trial, the investigational treatment may be started immediately on the subject if the available scientific and medical data indicate that the investigational product can be used to save the life of the trial participant, restore their health or relieve their suffering.

• Informed consent from the participant or their legal representative will be obtained as soon as possible and according to the deadlines defined by the CNERSH.

- Furthermore, the following conditions must be met [4]:

    i. the data subject does not express in an identifiable manner, verbally or through particular behavior, his or her refusal of the processing linked to the research project;
    ii. a doctor not associated with the research project will be consulted before the inclusion of a subject in the study in order to defend the interests of the potential participant;
    iii. the risks and constraints inherent to the clinical trial are minimal;
    iv. the expected results of the clinical trial may provide long-term benefit to people with the same disease or disorder, or whose health status is comparable.

(2) Clinical trials carried out on minors

- In a research situation involving minor subjects (aged under 21 years) [16], suffering from an illness that can be treated by the research product, the parents or legal guardians of the subject must be informed of the nature, the importance and implications of the clinical trial, and must give their authorization through informed, written and voluntary consent. In the case of minors (between 12 and under 21), assent is also required. This authorization can be revoked without prejudice at any time orally or in writing by the legal guardian or parent, who will decide whether we can continue to use the data collected during the trial.
- The use of the research product is in accordance with scientific and medical discoveries and has the objectives of: saving the life of the minor
- concerned, restore his health, relieve his suffering or prevent an illness.

- The clinical trial must directly benefit the group of patients suffering from the same illness as the minor concerned.
- Research must be absolutely necessary to confirm data obtained in clinical trials on other people or through other research methods.
- The clinical trial must relate to a clinical condition from which the minor concerned suffers or could suffer.
- The minor's participation in the clinical trial can only entail minimal risk and burden for him or her, which will be assessed by the competent ethics committee depending on each clinical trial and the type of research product.

(3) Clinical trials carried out on an incapable adult

In a situation where the potential subject participating in the trial is an adult incapable of understanding the issues of their participation in the clinical trial and of giving their consent voluntarily and in an informed manner:

- the use of the research product can only be justified on the basis of up-to-date and valid scientific and medical data and is intended to save the life of the trial participant and restore their health or relieve their suffering;
- the said trial must be directly related to and focused on a potentially life-threatening or significantly debilitating clinical condition suffered by the trial participant, and it should only involve a limited foreseeable risk to the participant;
- the risks incurred by the trial participant must be specifically defined in the trial protocol and continuously monitored by the investigator;
- the clinical trial can only be conducted if it can be expected and justified that the benefits of using the investigational product for the trial participant outweigh the risks involved;
- consent is given by the legal representative of the potential participant after having been duly informed in accordance with the general requirements defined in section 2 of chapter II;

- said clinical trial must be absolutely necessary to confirm data obtained from clinical trials conducted on persons capable of giving informed consent or through other research methods;
- With the exception of adequate compensation, no special benefits can be granted to the trial participant or their legal representative.

of discernment, the clinical trial may continue unless a request for the participant to withdraw from the clinical trial is expressed by his legal representative, or by the participant himself when he has regained his capacity.

(5) Clinical trials carried out on pregnant women, the embryo, or the fetus in vivo

- In the case of a clinical trial involving a pregnant woman, an embryo, or an in vivo fetus, the following requirements must be met: the ratio between the foreseeable risks and constraints for the pregnant woman, the embryo, or the in vivo fetus on one hand, and the expected benefit on the other hand, is not deemed disproportionate by the CNERSH; the risks and constraints inherent to the project are minimal for the embryo or in vivo fetus;
- the project makes it possible to expect essential results that could provide long-term benefit to other pregnant women or other embryos or fetuses in vivo.

(6) Clinical trials carried out on the embryo in vitro

In the event of a clinical trial on an in vitro embryo, the following requirements must be respected:

- the prior consent of the two (02) members of the couple or of the surviving member of the couple from which the embryos were derived is necessary, this consent is revocable at any time and without reason by one of the two parents as long as the research has not been completed. started;

- the embryos used in the research project must be conceived in vitro as part of medically assisted procreation and no longer the subject of a parental project;
- the parental couple or the surviving member of the couple must be informed of the nature of the planned research in order to enable them to give free and informed consent.

Section 4: Investigators Qualifications

(1) The principal investigator is responsible for conducting the clinical trial. He must reside in Cameroon and must have appropriate education, training and qualifications in ethics and science, including doctors of all specialties, pharmacists and oral and dental doctors.

(2) Basic scientists and paramedical staff, depending on the area of the clinical trial, may be combined to form the multidisciplinary team.

(3) The principal investigator must have appropriate training and qualifications in ethics and science.

(4) Clinical trials involving patients or healthy volunteers require the supervision of a physician or other qualified and competent healthcare professional.

(5) All the investigators involved in a clinical trial as well as the trial monitor must have official and documented training in Good Clinical Practices (GCP) dating back less than two years.

Section 5: Research product

(1) Research products (including active comparators and placebo if applicable) intended for clinical trials must be manufactured in accordance with internationally recognized Good Manufacturing Practice (GMP) principles [17].

(2) All documentation relating to Good Manufacturing Practices must be submitted by the sponsor or principal investigator to the competent authority.

(3) All investigational medicinal product must be subject to quality control by a national public or private pharmaceutical quality control laboratory approved by the competent authority.

(4) The laboratory that carries out quality control of the investigational medicinal product must be independent of the manufacturer.

(5) Quality control must take into account on the one hand the elements of the GMP documentation, and on the other hand compliance with the specifications of the finished product and the examination of the final packaging.

(6) Research products intended for clinical trials must be correctly labeled and the packaging must sufficiently identify:

- the clinical trial concerned;
- the medication(s) to be used;
- the identification number of the trial participant to whom the drug is to be administered;
- the name and address of the site where the clinical trial is being conducted;
- instructions on how this medicine should be used;
- the date of issue, if applicable;
- storage conditions;
- the expiration or expiry date, as applicable;
- the reference number, if applicable; And
- any other information required by the Ministry in charge of Public Health.

(7) Any experimental research product must benefit from a Temporary Authorization for Use (ATU) issued by the Ministry of Public Health before any use on trial participants. The ATU request for research products addressed to the Minister in charge of Public Health includes the following elements:

- the marketing authorization, or special import authorization/exemption issued where applicable;

- samples in sufficient number according to the procedures of the approved pharmaceutical quality control laboratory;
- the certificate of analysis of the samples issued by the manufacturer;
- the product manufacturing file;
- the clinical trial research protocol;
- the fees payable for quality control.

(8) When the research product must be imported into Cameroonian territory, the principal investigator is subject to request either an import authorization if the drug has a marketing authorization in Cameroon, or a special import exemption. otherwise. The said request addressed to the Minister in charge of Public Health includes the following elements:

- the stamped letter of request addressed to the Minister in charge of Public Health, setting out the reasons for importing the medicine;
- the product shipping slip signed by the pharmacist providing information on the total quantity, batch number and expiry dates of the products, names and addresses of the sender and recipient;
- the manufacturer's operating license or opening authorization;
- the GMP certificate;
- the free sale certificate;
- the copy of the marketing authorization for the medicinal product in other countries as well as the latest periodic safety update report or equivalent documents if applicable;
- the summary of product characteristics;
- mock-ups of the primary and secondary container labels;
- the research protocol;
- Administrative Research Authorization;
- ethical clearance.

(9) Destruction operations for research products must be documented according to the rules of the National Guide for the Management and Destruction of Medicines [18].

(10) If before, during or after a trial, the sponsor/promoter-investigator wishes to export the research product, the latter must obtain an export authorization from the Ministry in charge of Public Health [19].

Section 6: Management of biological samples

(1) The sponsor or his legal representative, the promoter-investigator or the principal investigator is required to provide proof of expertise and necessary technical capacity for the analysis of biological samples collected during clinical trials in Cameroon.

(2) The sponsor or their legal representative, the sponsor-investigator, or the principal investigator is required to involve Cameroonian expertise for the analysis of biological samples.

(3) In exceptional cases where it is necessary to export the biological samples out of Cameroon:

- A duly signed Materials Transfer Agreement by one or more research institutions within and outside Cameroon must be provided to the Ministry responsible for Public Health by the principal investigator. The agreement will specify the conditions of transfer, storage, sharing, use, and eventual destruction of the samples and derived products.
- The principal investigator must obtain an export authorization from the relevant public health authority justifying the need for the material transfer.
- The sponsor or their legal representative must submit reports to the Ministry responsible for health regarding the analyses of the biological materials exported out of Cameroon, including any scientific publications.

Section 7: Evaluation Models for Clinical Trial Protocols

(1) Following the submission of the Administrative Authorization for Research (AAR) application dossier:

The Regional Health Delegation must inform the principal investigator in writing about the completeness or administrative inadmissibility of their application dossier within a period of 72 working hours from the receipt of the clinical trial request by the Regional Health Delegation. application within 72 working hours from receipt of the clinical trial request by DROS;

- once notified, the principal investigator has 10 working days to complete his file;
- DROS has 21 days [20] to evaluate the clinical trial AAR application file from the date of submission of a complete file. This duration excludes the time taken by the principal investigator to answer the DROS questions;
- during the administrative evaluation, if modifications must be made to the elements of the application file, the principal investigator has a maximum period of 30 working days to do so. After this period, the request is considered inadmissible.

(2) DROS may, when evaluating a clinical trial application, recognize and/or use decisions, reports or information it deems relevant from other national regulatory authorities or regional or international bodies.

(3) DROS may establish expert advisory committees for the review of applications for administrative authorization of clinical trials and for any other questions arising after approval of the trial.

(4) All clinical trials conducted in Cameroon must be registered in a public local and/or international database (PACTR[2] at least) by the promoter/promoter-investigator and registered by the competent services in the registers of the National Research Ethics Committee for Human Health and the Ministry of Public Health maintained by the DROS.

(5) The list of approved or rejected clinical trial applications as well as the summary of evaluation reports and the status of approved clinical trials must be publicly accessible and periodically updated by DROS.

(6) Once contacted by the promoter/promoter-investigator, the Ethics Committee has a period of thirty (30) days, from the date of submission of the ethical clearance request file, to make a decision. However, in the event of a health emergency, the said deadline may be reduced to fifteen (15) days.

(7) The information letters emanating from the evaluation of the research protocol are notified to the principal investigator by the National Ethics Committee and the Ministry in charge of Public Health contained in the Guide entitled "Organization, operation and procedures of the system ethical and administrative evaluation of human health research in Cameroon.

(8) The presentation models of the protocol, the investigator's brochure, the biological material transfer agreement, and the study data sharing agreement will comply with Good Clinical Practice standards.

(9) The procedures for evaluating research protocols are carried out according to standardized operating procedures of the National Ethics Committee and the Ministry in charge of Public Health.

Section 8: Elements of refusal of ethical clearance or Administrative Research Authorization

A request for ethical clearance or AAR may receive an unfavorable opinion in the following cases:

- the required formalities are not completed (see sections 2, 3, 4, and 5 of Chapter II above);
- the competent services of the Ministry in charge of Public Health are in possession of information indicating that the trial site, the methodological approach, the research product or the research team are not suitable for the conduct of the clinical trial;
- for any other reasons deemed relevant by the Ministry of Public Health and/or the Ethics Committee.

Section 9: Recourse in the event of an unfavorable opinion

The promoter or his legal representative may contest an unfavorable decision rendered by the CNERSH or the Ministry of Public Health. To this end, the promoter or his legal representative must contact the competent authority (President of CNERSH/Minister in charge of Public Health) within a maximum period of thirty (30) days from notification of the disputed decision. The competent authority must respond to the principal investigator within sixty (60) days from the date of referral.

Section 10: Monitoring and Inspection of Clinical Trials

(l) All approved clinical trials must be passively monitored and continuous active from the date of approval until the end of the research.

(3) Following a monitoring and/or inspection mission by the competent services of the Ministry in charge of Public Health, the following measures may be taken:

- favorable opinion on the continuation of the clinical trial;
- request for improvement of one or more procedures in the conduct of the clinical trial within a specific time frame;
- request for additional information;

- application of the sanctions provided for by the legislation in force.

(4) The principal investigator will be notified in writing of the action taken at the end of the monitoring and/or inspection mission.

(5) A monitoring report (DROS, CNERSH, DPML) or inspection report (IGSPL) is sent to the Minister responsible for Public Health within 7 days following the mission.

Section 11: Reporting of adverse events to health products

During the implementation of the clinical trial, the investigator is required to notify systematically all adverse events that occur in participants in the CNERSH, DROS and DPML.

(1) Before the start of the clinical trial, the principal investigator is required to submit a copy of the AAR and the Ethical Clearance to the DPML.

(2) Serious adverse reactions are reported immediately (within 24 hours of the occurrence of the event).

(3) Minor adverse effects are reported as soon as possible.

(4) The delivery of adverse event notification forms is done according to the national notification circuit (Health District, Regional Public Health Delegation, MINSANTE Central Services), and to the email address minsantedros@yahoo.com and vigilancedpmlcmr(g)gmail.com.

Section 12: Clinical trial reports provided by the principal investigator

(1) The investigator principal or his legal representative must inform DROS in writing of the exact date of the start of the clinical trial (date of the first visit of the first participant included in the trial) within 90 days, and of the end of the clinical trial. clinical trial within 30 days following the last visit of the last participant.

(2) The principal investigator or his legal representative must submit to the DROS and the CNERSH semi-annual progress reports and a final summary report of the clinical trial which must include the certificate of destruction of the research product duly completed if applicable. This report must be submitted within one year after completion of the trial.

(3) if the trial is suspended or terminated prematurely by the sponsor or his legal representative, the principal investigator must communicate the reason in writing to the DROS and the national research ethics committee within 10 working days following the date of termination of the trial clinical.

(4) Of the feedback meetings must be organized by the principal investigator and his legal representative involving all study stakeholders, under the coordination of the DROS.

Section 13: Special requirements for clinical trials of Improved Traditional Medicines (ATMs)

(1) Clinical trials are required for improved traditional medicines in categories 3 and 4. A clinical trial may be required for improved traditional medicines in category 2 depending on the assessment of the DPML, the DROS and/or the CNERSH. The data obtained from these tests constitute an element of the Marketing Authorization file.

(2) The category of MTA is determined by the DPML according to the terms defined by the regulations in force.

(3) The Administrative Research Authorization application file for the clinical trial includes the following documents:

- a stamped request at the current rate addressed to the Minister in charge of Public Health;
- an authorization request form duly completed and signed by the principal investigator;
- the clinical trial protocol;
- the CV of the investigator(s);
- the participant information note and informed consent form;
- the GMP certificates of the drug manufacturing site(s) if applicable.
- declaration forms completed and signed by the investigators;
- proof of ethics committee approval;
- proof of payment of application fees;
- guaranteeing insurance coverage for subjects and investigators (multi-risk institutional insurance);
- the budget of the study;
- proof of availability of funding to conduct the study.

(4) Additional elements that may complete the application for ethical clearance and/or administrative research authorization may be required by the CNERSH or the DROS respectively.

(5) The DROS has a maximum period of 21 days for notification of the decision to grant, postpone or refuse authorization for the clinical trial.

(6) The DROS must set up a multidisciplinary technical committee of experts to examine applications for Administrative Authorization for clinical trials relating to improved traditional medicines.

(7) The terms of creation and operation of the technical committee of experts will be the subject of written procedures developed by DROS.

REPUBLIQUE DU CAMEROUN
Paix-Travail-Patrie

MINISTERE DE LA SANTE PUBLIQUE

REPUBLIC OF CAMEROON
Peace-Work-Fatherland

MINISTRY OF PUBLIC HEALTH

DECISION N° 3498 / D/ MINSANTE/SG/DROS/DAJC DU 04 OCT 2023
rendant exécutoire les Lignes Directrices pour les Essais Cliniques au Cameroun.

LE MINISTRE DE LA SANTE PUBLIQUE,

Vu la Constitution ;
Vu la loi n°96/03 du 04 janvier 1996 portant loi cadre dans le domaine de la santé ;
Vu la loi n°2022/008 du 27 Avril 2022 relative à la recherche médicale impliquant la personne humaine au Cameroun ;
Vu le décret n°2011/408 du 9 décembre 2011 portant organisation du Gouvernement, modifié et complété par le décret n°2018/190 du 02 mars 2018 ;
Vu le décret n° 2013/093 du 03 avril 2013 portant organisation du Ministère de la Santé Publique ;
Vu le décret n° 2019/002 du 04 janvier 2019 portant réaménagement du Gouvernement ;
Vu l'arrêté N° 977/A/MINSANTE/SESP/SG/DROS du 18 avril 2012 portant création, organisation et fonctionnement des Comités d'Ethique de la Recherche pour la Santé Humaine au sein des structures relevant du ministère en charge de la santé publique ;
Vu l'arrêté N° 1737/ A/MINSANTE/SG/DROS du 28 novembre 2013 fixant les modalités des frais d'évaluation des protocoles de recherche soumis aux comités d'éthique de la recherche pour la santé humaine pour l'obtention d'une clairance éthique ;

Considérant les nécessités de service,

DECIDE:

ARTICLE 1.- La présente décision rend exécutoire les lignes directrices pour les essais cliniques au Cameroun.

ARTICLE 2.- Les lignes directrices susvisées sont jointes en annexe de la présente décision.

ARTICLE 3.- Les lignes directrices pour les essais cliniques au Cameroun feront l'objet d'une actualisation en tant que de besoin, à l'initiative du Ministre chargé de la Santé Publique.

ARTICLE 4.- La présente décision qui prend effet à compter de la date de signature, sera enregistrée puis communiquée partout où besoin sera, en français et en anglais.-/

Yaoundé, le _____

REPUBLIQUE DU CAMEROUN Republic of Cameroon PAIX-TRAVAIL-PATRIE Peace-Work-Fatherland LE MINISTRE The Minister Ministry of Public Health MINISTERE DE LA SANTE PUBLIQUE

LE MINISTRE DE LA SANTE PUBLIQUE,

Dr. MANAOUDA Malachie